

Health & Safety Requirements for Subcontractors

The Subcontractor is required to provide, together with its offer, the following information and documentation.

1. Health & Safety Gouvernance

- H&S Policy statement, including “Alcohol and Drug” policy
- Company organization chart and Project organisation chart, specifying H&S positions and roles
- H&S Management System Manual (table of contents)
- Copy of H&S Certification (ISO45001...), if any

2. Health and Safety Management

- Typical or project specific H&S Plan that addresses the following, but not limited to :
 - H&S objectives, targets
 - List of H&S procedures applicable to the scope of work
 - Safety Culture program including management leadership
 - H&S responsibilities
 - H&S communication
 - H&S induction and training program
 - Protocol for periodical medical fitness examination
 - Risk Assessment procedure
 - Provision and maintenance of Personal Protective Equipment (PPE) – standard and required for specific activities
 - Description of management of construction plants and equipment used with reference to registration, control and maintenance in a safe working condition.
 - H&S inspections and audit
 - Accident / Incident investigation and reporting
 - Emergency Preparedness and Response procedures

3. Health and Safety Past Performances

- List of the last three years' safety statistics within Subcontractor's organisation according to the following definitions. Statistics are based on Subcontractor's own employees, Subcontractor's subcontracted employees and temporary/agency personnel

KPI	Definition
Worked manhours	Cumulative number of hours spent at work for a given period
Fatalities (FAT)	The victim passed away following the accident
Lost Time Injuries (LTI)	The accident resulted in a minimum of 1 day off, with the employee not resuming work at the resumption of the next post normally scheduled
Lost Time Injury Rates (LTIR)	$= (LTI + FAT) * \text{Worked manHours} / 1 \text{ million}$
Work Restricted Cases (WRC)	Occupational Injury for which the victim returns to work on the date and time planned, but in a lightened, arranged or different activity
Medical Treatment Cases (MTC)	Occupational Injury for which the victim goes to seek a medical exam outside of the place of work, then returns to work at the dates and times scheduled in his normal activity
First Aid Cases (FAC)	Occupational Injury for which the victim receives first care at his place of work, without external consultation, and resumes his usual work
Occupational Health Diseases	Disease resulting from occupational exposure to health- damaging conditions or substances
Near-Misses (NM)	A Near Miss is an unexpected and sudden event in which no personal injury was sustained, but where, given a slight shift in time or position, injury easily could have occurred
Severe events	The following injuries are considered as severe: 1. Fatal Injuries 2. Injuries resulting in irreversible impacts: permanent physical incapacity and/or disability (e.g. functional loss of a sense, loss or paralysis of a limb, loss or dysfunction of an organ) 3. Injuries with any of the following consequences, even if reversible: - Serious burns requiring surgery or a skin graft - Victim in a coma, and/or requiring life-saving emergency treatment, or placed in an intensive care unit - Fracture / Internal hemorrhage / Organ injury / Poisoning, requiring inpatient hospitalisation for more than 48hrs continuously: the 48hrs period runs from the moment when the medical profession defines that the care requires hospital accommodation
High Potential events (HiPo)	Any event which could have led to a Severe Occupational Injury or a Property Damage with significant consequences

- Explanation of the most significant injuries that happened over the last 3 years (Investigation Report, Safety Flash...)
- Description of corrective actions undertaken to avoid recurrence of reported injuries
- Any other proactive indicator monitored by the Subcontractor